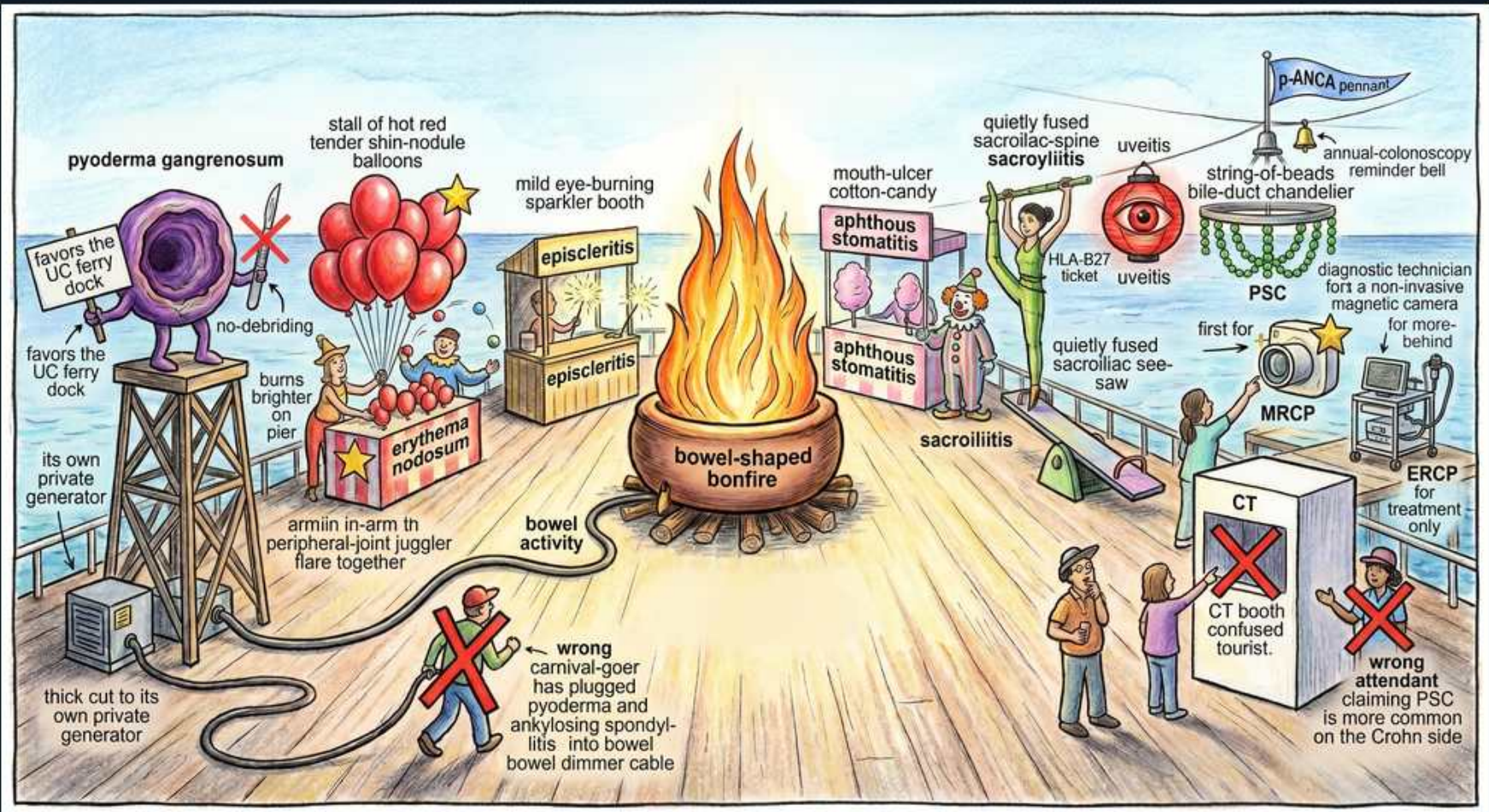


IBD — Extraintestinal Manifestations



1. Bowel-shaped bonfire (activity)

WHAT YOU SEE

Central bonfire in a bowel-shaped pit labeled 'BOWEL ACTIVITY'.

WHAT IT ENCODES

The fire = bowel disease ACTIVITY. EIMs that 'burn brighter with the fire' track bowel activity and improve when the gut flare is treated; those on their 'own private generator' run independently of bowel activity.

2. Erythema nodosum balloons (activity, star)

WHAT YOU SEE

Bunch of red shin-nodule balloons with a gold star, 'ERYTHEMA NODOSUM', 'burns brighter on pier'.

WHAT IT ENCODES

Erythema nodosum = tender red shin nodules, TRACKS bowel activity (more common in Crohn's), resolves as the flare is controlled. Most common cutaneous EIM of IBD.

3. Peripheral arthritis jugglers (activity)

WHAT YOU SEE

Two performers arm-in-arm, 'peripheral-joint juggler flare together'.

WHAT IT ENCODES

Peripheral (large-joint, type 1 pauciarticular) arthritis is non-erosive and TRACKS bowel activity — flares with the gut and settles with bowel treatment. Distinct from axial arthritis which is independent.

4. Episcleritis sparkler booth (activity)

WHAT YOU SEE

Booth 'mild eye-burning sparkler' labeled 'EPISCLERITIS'.

WHAT IT ENCODES

Episcleritis = mild, non-painful red eye that TRACKS bowel activity and resolves with flare control. Contrast with uveitis (painful, vision-threatening, activity-INDEPENDENT).

5. Aphthous stomatitis cotton candy (activity)

WHAT YOU SEE

Cotton-candy booth 'mouth-ulcer' labeled 'APHTHOUS STOMATITIS'.

WHAT IT ENCODES

Aphthous oral ulcers TRACK bowel activity (especially Crohn's, mouth-to-anus involvement) and improve with gut treatment.

6. Pyoderma gangrenosum — no debridng (red X)

WHAT YOU SEE

Top-left purple ulcer with scissors crossed by red X, 'no-debridng', 'favors UC ferry dock'.

WHAT IT ENCODES

Pyoderma gangrenosum = rapidly enlarging painful ulcer with violaceous undermined border, more common in UC, runs INDEPENDENT of bowel activity. NEVER debride/surgically trim — debridement triggers pathergy and worsens it. Treat with steroids/immunosuppression.

BOARD TRAP

"Debride pyoderma gangrenosum" → wrong; debridement causes pathergy and worsens it; treat medically.

7. PG/AS on own generator (independent)

WHAT YOU SEE

Air-conditioner units 'its own private generator', 'thick cut to its own private generator'.

WHAT IT ENCODES

Pyoderma gangrenosum and axial arthritis/ankylosing spondylitis are ELECTRICALLY SEPARATE — they do NOT track bowel activity and persist even after the gut is quiet (or post-colectomy). Manage on their own merits.

8. Sacroiliitis / AS fused see-saw (HLA-B27)

WHAT YOU SEE

Fused/stiff see-saw figures 'quietly fused sacroiliac', 'SACROILIITIS', 'HLA-B27 ticket'.

WHAT IT ENCODES

Axial arthritis = sacroiliitis / ankylosing spondylitis, HLA-B27-associated, INDEPENDENT of bowel activity. Morning stiffness improving with exercise; treat with PT and anti-TNF (NSAIDs cautiously as they can flare the gut).

9. Uveitis lanterns (independent)

WHAT YOU SEE

Red lantern booths labeled 'UVEITIS'.

WHAT IT ENCODES

Uveitis = painful red eye, photophobia, blurred vision — vision-threatening and INDEPENDENT of bowel activity (HLA-B27 linked). Urgent ophthalmology referral; contrast with benign activity-linked episcleritis.

10. PSC string-of-beads chandelier (activity-independent)

WHAT YOU SEE

Beaded duct chandelier labeled 'PSC', 'string-of-beads bile-duct'.

WHAT IT ENCODES

Primary sclerosing cholangitis = beaded intra/extrahepatic ducts, p-ANCA positive, strongly UC-associated and INDEPENDENT of bowel activity. Raises cholangiocarcinoma and colorectal cancer risk → needs ANNUAL colonoscopy.

11. MRCP first (gold star)

WHAT YOU SEE

Imaging technician at a magnetic camera 'first for' labeled 'MRCP', gold-star context.

WHAT IT ENCODES

MRCP (non-invasive magnetic imaging) is FIRST-LINE to diagnose PSC — shows the beaded strictures without procedural risk. Gold star = correct first test.

12. ERCP treatment only

WHAT YOU SEE

Far-right label 'ERCP for treatment only'.

WHAT IT ENCODES

ERCP in PSC is reserved for TREATMENT (dilating dominant strictures, stenting, brushings for malignancy) — not first-line diagnosis. It carries pancreatitis/cholangitis risk, so don't lead with it.

BOARD TRAP

"Use ERCP first to diagnose PSC" → wrong; MRCP is first-line; ERCP is for treatment of dominant strictures.

13. Annual-colonoscopy reminder bell

WHAT YOU SEE

Bell/pennant top-right 'annual-colonoscopy reminder bell', 'p-ANCA pennant'.

WHAT IT ENCODES

PSC + UC patients need ANNUAL surveillance colonoscopy from diagnosis due to markedly elevated colorectal cancer risk. p-ANCA marks the UC/PSC association.

14. Wrong attendant: PSC more on Crohn side (red X)

WHAT YOU SEE

Bottom-right crossed-out attendant 'wrong, claiming PSC is more common on the Crohn side'.

WHAT IT ENCODES

FALSE: PSC is associated with ULCERATIVE COLITIS, not predominantly Crohn's. The misattribution to the Crohn side is the explicit board trap.

BOARD TRAP

"PSC is more common on the Crohn's side" → wrong; PSC is UC-associated.

15. Wrong carnival-goer plugs PG/AS into activity (red X)

WHAT YOU SEE

Bottom-center figure crossed out 'wrong... plugged pyoderma and ankylosing spondylitis into bowel dimmer cable'.

WHAT IT ENCODES

FALSE: treating pyoderma gangrenosum and ankylosing spondylitis as if they track bowel activity. They are activity-INDEPENDENT and persist regardless of gut control — a classic trap.

BOARD TRAP

"Pyoderma gangrenosum and ankylosing spondylitis track bowel activity" → wrong; they are independent of bowel activity.